Medicare Open Enrollment 2027 Decision Guide

Pick the right 2027 coverage before the deadline: plain-English Advantage vs Medigap vs Part D, late-penalty explainer, plan-comparison worksheet, key-dates checklist. Guidance only - confirm plan details at medicare.gov.

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Inside this guide

10 chapters · Medicare Open Enrollment 2027 Decision Guide

1. Chapter 1: What Changes on January 1, 2027 — and What Doesn't
2. Chapter 2: Your Enrollment Window: Dates, Deadlines, and What Happens If You Miss Them
3. Chapter 3: Medicare Advantage vs. Original Medicare + Medigap: A Plain-English Comparison
4. Chapter 4: Reading a Part D Drug Plan: Formularies, Tiers, and Pharmacy Networks
5. Chapter 5: The Late-Enrollment Penalty Explained: How It's Calculated and How to Avoid It
6. Chapter 6: Checking Whether Your Doctors, Hospitals, and Drugs Are In Network for 2027
7. Chapter 7: The Plan-Comparison Worksheet: Scoring Cost, Coverage, and Convenience Side by Side
8. Chapter 8: Switching Plans Mid-Enrollment: Rules for Changing Advantage, Medigap, and Part D
9. Chapter 9: The Key-Dates Checklist: A Week-by-Week Plan Through December 7, 2026
10. Chapter 10: After You Enroll: Confirming Coverage, Keeping Records, and Where to Get Free Help

CHAPTER 1

Chapter 1: What Changes on January 1, 2027 — and What Doesn't

Most of what you'll read about the 2027 plan year is plan-specific, not law-specific. Before you compare a single plan, separate the two: rules that apply to everyone, and details that differ by plan and by your county.

What doesn't change

- The enrollment calendar. The Annual Enrollment Period for coverage starting January 1, 2027 runs October 15 through December 7, 2026. (Chapter 2 covers dates in detail.)
- The structure of your choices. Original Medicare (Part A and Part B) plus optional Part D and Medigap, or a Medicare Advantage plan. Both remain available.
- Guaranteed issue in Medigap. Your one-time Medigap open enrollment window still depends on when you first enrolled in Part B at 65 or older — not on the calendar year.
- Plan-year resets. Deductibles, out-of-pocket limits, and formularies reset on January 1 for every plan. This is normal, not a change.

What to verify for 2027

1. Your plan's Annual Notice of Change (ANOC). It should arrive by late September 2026. Read the drug and pharmacy sections first; those change most often.
2. Your premium and deductible for 2027. Premiums and cost-sharing are set per plan, per year. Assume nothing carries over.
3. Formulary and tier placement for each drug you take. A drug can move tiers without leaving the formulary.
4. Provider and pharmacy networks for the 2027 plan year.
5. Any federal parameters (Part B premium, Part D parameters) — confirm figures at SSA.gov and

Medicare.gov when published, not from memory or social media.

Your first action

Put the ANOC in one folder and mark three dates on your calendar: its arrival, October 15, and December 7. Everything else builds on those.

CHAPTER 2

Chapter 2: Your Enrollment Window: Dates, Deadlines, and What Happens If You Miss Them

The annual Medicare Open Enrollment (AEP) runs October 15 through December 7 each year, and that window applies to the 2027 plan year as well. Plan changes you make during this period take effect January 1, 2027.

The dates that matter

- October 1: Plans release their next-year details. Marketing can begin, and you can start comparing.
- October 15: Enrollment opens. You can join, switch, or drop a Medicare Advantage or Part D plan.
- December 7: Enrollment closes at 11:59 p.m. local time. This is the last day to make a change for January 1.
- December 8 – December 31: You cannot enroll or switch during this gap.
- January 1, 2027: New coverage begins.

What happens if you miss December 7

You are not locked out for the whole year, but your options narrow:

- Medicare Advantage Open Enrollment Period (January 1 – March 31, 2027): If you are already in a Medicare Advantage plan, you can switch to another Advantage plan or return to Original Medicare once. You can add a Part D plan when you return to Original Medicare.
- Special Enrollment Periods: Certain life events (moving, losing employer coverage, a plan leaving your area) can open a window. You generally must act within a set number of days of the event.
- Original Medicare + Medigap: If you stay in Original Medicare, you can enroll in a Part D plan during a valid SEP or the next AEP. Medigap has its own rules and is not bound to the AEP dates.

Action steps

1. Mark October 15 and December 7 on your calendar now.
2. Confirm your current plan's 2027 details after October 1.
3. Submit any change by December 7 and save the confirmation number.

CHAPTER 3

Chapter 3: Medicare Advantage vs. Original Medicare + Medigap: A Plain-English Comparison

The choice comes down to two structures. Original Medicare (Parts A and B) is run by the federal government; you add a Part D drug plan and, optionally, a Medigap policy to fill gaps. Medicare Advantage (Part C) is a private plan that replaces the delivery of Parts A and B and usually bundles drugs. You cannot use Medigap to pay Advantage costs.

Compare on these five points

1. Provider access. Original Medicare lets you see any provider who accepts Medicare nationwide, no referral needed for specialists. Advantage plans use networks (HMO, PPO); out-of-network care costs more or isn't covered. Check your doctors and hospital before deciding.
2. Cost structure. Original Medicare charges the Part B premium plus coinsurance with no annual cap unless you hold Medigap. Advantage plans charge the Part B premium plus plan premiums, copays, and an in-network annual out-of-pocket maximum.
3. Drug coverage. Advantage usually includes Part D; Original Medicare requires a standalone Part D plan. Either way, verify your drugs on the formulary (Chapter 4).
4. Prior authorization. Advantage plans commonly require approval for certain services and drugs. Original Medicare generally does not.
5. Flexibility to change. You can switch Advantage plans or return to Original Medicare during Open Enrollment, but buying Medigap later can involve medical underwriting in most states.

Decision test

- Want broad provider choice and predictable costs via Medigap? Lean Original Medicare + Medigap + Part D.

- Prefer lower premiums and bundled extras, and your doctors are in-network? Lean Advantage.
- Take multiple expensive drugs or see specialists often? Run the total annual cost for both before choosing.

Write down your doctors, drugs, and travel patterns first; they drive the answer more than premiums alone.

CHAPTER 4

Chapter 4: Reading a Part D Drug Plan: Formularies, Tiers, and Pharmacy Networks

A Part D plan is three lists stacked on top of each other: which drugs it covers (formulary), what you pay per drug (tiers), and where you can fill them (pharmacy network). A plan that looks cheap on the summary page can cost you hundreds more at the counter if your drug sits on a high tier or your pharmacy is out of network.

Step 1: Check the formulary for your exact drugs

- Pull every prescription you take, including dose and quantity per month.
- Open the plan's formulary (drug list) PDF and search each drug by its generic name, not the brand name.
- Note the status: on formulary, on formulary with restrictions, or not covered. Restrictions include prior authorization, step therapy, and quantity limits. A "covered" drug with prior authorization still needs your doctor to submit paperwork before the plan pays.
- If a drug is missing, ask the plan whether it has a formulary exception process and how long it takes.

Step 2: Map each drug to its tier

- Find the tier number next to each drug (typically 1 through 5).
- Match the tier to the plan's copay or coinsurance for that tier. A percentage coinsurance on a high-tier drug can cost far more than a flat copay.
- Flag any drug on tier 3 or above. These are where the cost surprises live.

Step 3: Confirm your pharmacy is in network

- Check whether the plan uses preferred pharmacies with lower cost sharing and standard pharmacies with higher cost sharing.

- Verify your regular pharmacy appears in the 2027 directory, not last year's.
- Check whether mail order is required for maintenance drugs or simply offered at a discount.

Step 4: Run the math

Total your estimated annual cost: premiums plus per-fill costs for each drug at your actual pharmacy. Compare that number across plans, not the premium alone.

CHAPTER 5

Chapter 5: The Late-Enrollment Penalty Explained: How It's Calculated and How to Avoid It

Two different penalties exist — one for Part B and one for Part D. They work differently, so handle each separately.

The Part B Penalty

- It adds 10% to your Part B premium for every full 12-month period you went without Part B after you were first eligible.
- The increase is permanent: it stays for as long as you have Part B.
- The penalty is based on the standard Part B premium, not on your income-related adjustment.
- You avoid it by enrolling during your Initial Enrollment Period (the 7-month window around your 65th birthday) or by qualifying for a Special Enrollment Period through active employer coverage.

The Part D Penalty

- It is 1% of the national base beneficiary premium multiplied by the number of full months you went without creditable drug coverage.
- Unlike Part B, this penalty is recalculated each year because the base premium changes.
- You avoid it by keeping creditable coverage — from an employer plan, a union plan, or another source — and by enrolling in Part D within two months of losing that coverage.
- If you have no drug coverage and no creditable alternative, enroll during Open Enrollment rather than waiting.

How to Check Before You Owe

1. Gather your Medicare card and any employer or union coverage notices.
2. Ask each plan whether its drug coverage is "creditable" in writing.
3. Call 1-800-MEDICARE or your State Health Insurance Assistance Program (SHIP) to confirm whether a penalty already applies to your record.
4. If a penalty was applied in error, request a review with documentation of your prior coverage.

Avoidance Checklist

- Enroll in Part B on time, or keep proof of active employer coverage.
- Keep creditable drug coverage continuous.
- Enroll within two months of losing creditable coverage.
- Save every creditable-coverage notice you receive.

CHAPTER 6

Chapter 6: Checking Whether Your Doctors, Hospitals, and Drugs Are In Network for 2027

Networks and formularies reset on January 1, 2027. A doctor who accepted your plan in 2026 can be out of network next year, and a drug on Tier 2 can move to Tier 3. Verify each item yourself against the 2027 plan documents before you enroll.

Doctors and Hospitals

1. Get the plan's 2027 provider directory. For Medicare Advantage, use the plan's own lookup tool and select the 2027 plan year if offered; otherwise request the printed 2027 directory.
2. Search each doctor by full name and NPI (National Provider Identifier) if the tool allows it. Names repeat; NPIs don't.
3. Call the doctor's office and ask two questions: "Are you in network for [plan name] in 2027?" and "Are you accepting new patients on this plan?" Directory listings go stale.
4. Check hospitals the same way, including any hospital where you have scheduled care. Confirm the facility and the physicians who practice there separately — hospital-based doctors (anesthesiologists, radiologists, hospitalists) often bill outside the hospital's network.
5. For Original Medicare, networks don't apply, but confirm your doctors accept Medicare assignment and whether they accept Medigap plans without billing you extra.

Drugs

1. Download the plan's 2027 formulary and search every drug you take by generic and brand name.
2. Note the tier and any prior authorization, step therapy, or quantity limit for each drug.
3. Confirm your pharmacy is in the 2027 network and whether it's preferred or standard cost-sharing.
4. If a drug is missing or restricted, ask the plan whether an exception process exists and what

documentation it requires.

Record each confirmation — who you spoke with, the date, and the answer — and keep it with your enrollment paperwork.

CHAPTER 7

Chapter 7: The Plan-Comparison Worksheet: Scoring Cost, Coverage, and Convenience Side by Side

A comparison worksheet turns a stack of plan brochures into a ranked list. Build one row per plan you're seriously considering, then score each plan on three categories.

Set Up the Worksheet

Create a spreadsheet or a paper grid with one column per plan. Use these row groups:

- Cost: monthly premium, annual deductible, primary-care copay, specialist copay, urgent-care copay, ER copay, and your best estimate of annual out-of-pocket maximum exposure.
- Coverage: each of your prescription drugs with its tier and your estimated annual cost; whether your doctors and hospital are in network; dental, vision, and hearing benefits if you use them.
- Convenience: pharmacy in network, mail-order option, whether referrals are required, prior-authorization rules for your drugs or conditions, and plan star rating.

Score Each Category

Use a simple 1–5 scale per category, where 5 is best:

- Cost score: rank plans by estimated total annual cost (premiums plus expected copays and drug costs). Give the lowest total a 5, the next a 4, and so on.
- Coverage score: start at 5 and subtract 1 for each of your drugs not on formulary or placed on a high tier, and 1 for each doctor or hospital out of network.
- Convenience score: start at 5 and subtract 1 for each friction point, such as a required referral, a prior authorization on a drug you take, or a pharmacy you don't use.

Decide

Add the three scores. Weight cost double if your budget is tight; weight coverage double if you have ongoing specialist care or expensive drugs. If two plans tie, choose the one with the lower out-of-pocket maximum. Keep the completed worksheet with your enrollment records so you can compare again next year.

CHAPTER 8

Chapter 8: Switching Plans Mid-Enrollment: Rules for Changing Advantage, Medigap, and Part D

Changing your mind after you enroll works differently depending on what you're moving between. The rules below apply during the October 15–December 7, 2026 window unless noted.

Switching within Medicare Advantage

- You can switch from one MA plan to another, or from MA back to Original Medicare, once during Open Enrollment. The new choice takes effect January 1, 2027.
- If you move from MA to Original Medicare, you are not guaranteed a Medigap policy. In most states, insurers can ask health questions and decline you. Before you drop MA, apply for Medigap first and confirm acceptance in writing.
- If you enroll in a new MA plan, your old one ends automatically on December 31. You don't need to cancel it.

Switching within Part D

- You can change standalone Part D plans once during Open Enrollment. The new plan replaces the old one on January 1.
- If you're in an MA plan that includes drug coverage (MA-PD) and want a standalone Part D plan instead, you must leave the MA plan. You can't hold both.

After December 7: the Medicare Advantage Open Enrollment Period

- From January 1 through March 31, 2027, you get one change if you're already in an MA plan: switch to a different MA plan, or drop MA and return to Original Medicare.

- This window does not let you add or switch a standalone Part D plan, and it does not restore Medigap guaranteed-issue rights.

Before you submit any change

1. Confirm your doctors and drugs are in the new plan's 2027 network and formulary.
2. Check whether the new plan's premium, deductible, and copays fit your Chapter 7 worksheet.
3. Save the confirmation number and date of every enrollment you submit.

CHAPTER 9

Chapter 9: The Key-Dates Checklist: A Week-by-Week Plan Through December 7, 2026

VERIFY BEFORE YOU ACT

Use this calendar to work backward from the December 7, 2026 deadline. The 2027 Annual Enrollment Period (AEP) runs October 15 through December 7, 2026. Confirm exact dates with Medicare.gov or 1-800-MEDICARE before relying on them.

Before October 15, 2026

- By September 30: Gather your current plan's Annual Notice of Change (ANOC) and Evidence of Coverage (EOC). Note every change to premiums, copays, formularies, and pharmacy networks.
- First week of October: List your doctors, specialists, and prescriptions (name, dose, frequency). This becomes your comparison input.

During AEP: October 15 – December 7, 2026

- Week 1 (Oct 15–21): Read the ANOC cover to cover. Flag anything that changed.
- Week 2 (Oct 22–28): Run your doctors and drugs through each candidate plan's 2027 network and formulary. Record "in network / out of network" and "covered / not covered" for each.
- Week 3 (Oct 29–Nov 4): Score plans on your worksheet. Narrow to two or three finalists.
- Week 4 (Nov 5–11): Re-check the two finalists against the plan's official 2027 documents. Confirm the pharmacy you use is in-network.
- Week 5 (Nov 12–18): Enroll. Do not wait for the final days — enrollment systems slow near the deadline.
- Week 6 (Nov 19–25): If you enrolled, save your confirmation number and date. If you're still deciding, set a hard stop of December 1.

- Final days (Dec 1–7): Last chance to enroll or change your mind. Changes made during AEP take effect January 1, 2027.

After December 7

- Keep your enrollment confirmation. Watch for your new plan card and member materials in the mail before January 1, 2027.

CHAPTER 10

Chapter 10: After You Enroll: Confirming Coverage, Keeping Records, and Where to Get Free Help

Confirm your enrollment in writing

1. If you enrolled through Medicare.gov or 1-800-MEDICARE, save the confirmation number the screen or representative gives you.
2. If you enrolled through a plan directly, ask for a confirmation letter and note the date you should expect it.
3. Within two weeks, check that the plan shows as active in your Medicare account. If it doesn't appear, call 1-800-MEDICARE before December 31, 2026.
4. When your new card arrives, check the plan name, member ID, and effective date (it should read January 1, 2027). Call the plan immediately if anything is wrong.

Keep a paper trail

- Save copies of your enrollment confirmation, the plan's Annual Notice of Change, and the Summary of Benefits.
- Write down the date, time, and name of anyone you speak with at Medicare or the plan, plus a reference number.
- Keep the receipts for any premium you pay in January. They prove your coverage started on time if a claim is later denied.
- Store everything in one folder, paper or digital, so you can find it during a billing dispute.

Where to get free help

- 1-800-MEDICARE (1-800-633-4227): free, 24/7, and the fastest way to check enrollment status.
- Your State Health Insurance Assistance Program (SHIP): free, unbiased counseling. Find your local office through Medicare.gov or your state's aging agency.
- The plan's member services line: free, but staff work for the plan, so use SHIP for comparisons and second opinions.

No legitimate counselor will charge you for enrollment help or ask for payment over the phone. If someone does, hang up and call 1-800-MEDICARE.